

Sulpin F.C. Tablet 200mg Sulpiride 200mg

Product Name

Sulpin F.C. Tablet 200mg

Product Description

It is a white round, biconvex film-coated tablet with “ S ”
200 on one side.

Composition

Each tablet contains Sulpiride 200mg.

Mode of action

Sulpiride is a member of the group of substituted benzamides, which are structurally distinct from the phenothiazines, butyrophenones and thioxanthenes. Current evidence suggests that the actions of Sulpiride hint at an important distinction between different types of dopamine receptors or receptor mechanisms in the brain. Behaviourally and biochemically, Sulpiride shares with these conventional neuroleptics a number of properties indicative of cerebral dopamine receptor antagonism. Essential and intriguing differences include lack of catalepsy at doses active in other behavioural tests, lack of effect in the dopamine sensitive adenylate cyclase systems, lack of effect upon noradrenaline or 5HT turnover, negligible anticholinesterase activity, no effect on muscarinic or GABA receptor binding and a radical difference in the binding of tritiated Sulpiride to striatal preparations in-vitro, compared to ³H-spiperone or ³H-haloperidol. These findings indicate a major differentiation between Sulpiride and conventional neuroleptics which lack such specificity.

Pharmacodynamics

Sulpiride is a highly selective dopamine antagonist with bi-modal dose-dependant and antidepressant and antipsychotic activity. Flurid schizophrenic symptoms respond to high doses of Sulpiride, whereas administration of lower doses results in mood elevation and alerting effect. Schizophrenia characterised by lack of social contact can benefit strikingly from treatment. Improvement of both flurid and negative symptoms occurs, often within the first few days of treatment. The sedation and blunted effects characteristically associated with classical neuroleptics of the phenothiazine or butyrophenone type are not features of Sulpiride therapy.

Pharmacokinetics

Sulpiride is slowly absorbed after oral dosing.

Peak sulpiride serum levels are reached 2-6 hours after an oral dose. The majority of studies have shown that the elimination half-life in plasma is 6-8 hours. Approximately 40% Sulpiride is bound to plasma proteins. The mean concentration of Sulpiride in the cerebrospinal fluid is 13% of its serum concentration. 95% of the compound is excreted in the urine and faeces as unchanged Sulpiride.

Indication

For the treatment of acute and chronic schizophrenia.

Recommended Dosage

Adults

A starting dose of 400mg to 800mg daily, given as one or two tablets twice daily (morning and early evening) is recommended.

Predominantly positive symptoms (formal thought disorder, hallucinations, delusions, incongruity of affect) respond to higher doses, and a starting dose of at least 400mg twice daily is recommended, increasing if necessary up to a suggested maximum of 1200mg twice daily. Increasing the dose beyond this level has not been shown to produce further improvement.

Predominantly negative symptoms (flattening of affect, poverty of speech, anergia, apathy, as well as depression) respond to doses below 800mg daily; therefore, a starting dose of 400mg twice daily is recommended. Reducing this dose towards 200mg twice daily will normally increase the alerting effect of Sulpin.

Patients with mixed positive and negative symptoms, with neither predominating, will normally respond to dosage of 400-600mg twice daily.

Children

Clinical experience in children under 14 years of age is insufficient to permit specific recommendations.

Elderly

The same dose ranges are applicable in the elderly, but the dose should be reduced if there is evidence of renal impairment.

Mode of Administration

Sulpin is administered by oral.

Contraindications

Sulpin Tablet is contraindicated in patient with:-

- Phaeochromocytoma and acute porphyria.
- Hypersensitivity to the active ingredient or to other ingredients of the drug.
- Concomitant prolactin-dependent tumours e.g. pituitary gland prolactinomas and breast cancer Association with levodopa

Warnings and Precautions

Warning

Increased motor agitation has been reported at high dosage in a small number of patients: in aggressive, agitated or excited phases of the disease process, low doses of sulpiride may aggravate symptoms. Care should be exercised where hypomania is present.

Extrapyramidal reactions, principally akathisia have been reported in a small number of cases. If warranted, reduction in dosage or anti-parkinsonian medication may be necessary.

As with other neuroleptics, neuroleptic malignant syndrome, a potentially fatal complication, which is characterised by hyperthermia, muscle rigidity, autonomic

instability, altered consciousness and elevated CPK levels, has been reported. In such an event, all antipsychotic drugs, including Sulpiride, should be discontinued.

Elderly patients are more susceptible to postural hypotension, sedation and extrapyramidal effects.

In patients with aggressive behaviour or agitation with impulsiveness, sulpiride could be given with a sedative.

Acute withdrawal symptoms, including nausea, vomiting, sweating and insomnia have been described after abrupt cessation of antipsychotic drugs. Recurrence of psychotic symptoms may also occur, and the emergence of involuntary movement disorders (such as akathisia, dystonia and dyskinesia) have been reported. Therefore, gradual withdrawal is advisable.

Precautions

In elderly patients, as with other neuroleptics, sulpiride should be used with particular caution

In children, efficacy and safety of sulpiride have not been thoroughly investigated. Therefore, caution should be exercised when prescribing to children.

When neuroleptic treatment is absolutely necessary in a patient with Parkinson's disease, sulpiride can be used, although caution is in order.

Sulpin induces slight EEG modifications only, and whilst neuroleptics may lower the epileptogenic threshold, this has not been evaluated with sulpiride. Caution is advised in prescribing it for patients with unstable epilepsy, and patients with a history of epilepsy should be closely monitored during therapy with sulpiride.

In patients requiring Sulpin who are receiving anti-convulsant therapy, the dose of the anti-convulsant should not be changed.

Cases of convulsions, sometimes in patients with no previous history, have been reported.

Sulpin has no significant anticholinergic effect. As with all drugs for which the kidney is the major elimination pathway, the dose should be reduced and titrated in small steps in cases of renal insufficiency.

Prolongation of the QT interval:

Sulpiride may induce a prolongation of the QT interval. This effect, known to potentiate the risk of serious ventricular arrhythmias such as torsade de pointes is enhanced by the pre-existence of bradycardia, hypokalaemia, congenital or acquired long QT interval.

Before any administration, and if possible according to the patient's clinical status, it is recommended to monitor factors which could favour the occurrence of this rhythm disorder

- bradycardia less than 55 bpm,
- electrolyte imbalance in particular hypokalaemia (which should be corrected),
- congenital prolongation of the QT interval.
- on-going treatment with a medication likely to produce pronounced bradycardia (< 55 bpm), hypokalaemia, decreased intracardiac conduction, or prolongation of the QTc interval (see section 4.5).

Sulpiride should be prescribed with caution in patients presenting with these factors and patients with cardiovascular disorders which may predispose to prolongation of the QT interval.

Avoid concomitant treatment with other neuroleptics

Stroke:

In randomised clinical trials versus placebo performed in a population of elderly patients with dementia and treated with certain atypical antipsychotic drugs, a 3-fold increase of the risk of cerebrovascular events has been observed. The mechanism of such risk increase is not known. An increase in the risk with other antipsychotic drugs or other populations of patients cannot be excluded. Sulpiride should be used with caution in patients with stroke risk factors.

Interactions with Other Medicaments

Levodopa: reciprocal antagonism of effects between levodopa and neuroleptics.

Alcohol: alcohol enhances the sedative effects of neuroleptics.

Avoid the consumption of alcoholic beverages and drugs containing alcohol.

Combination with the following medications which could induce torsades de pointes or prolong the QT interval:

- Bradycardia-inducing medications such as beta-blockers, bradycardia-inducing calcium channel blockers such as diltiazem and verapamil, clonidine; digitalics.
- Medications which induce electrolyte imbalance, in particular those causing hypokalaemia: hypokalaemic diuretics, stimulant laxatives, IV amphotericin B, glucocorticoids, tetracosactides.

Electrolyte imbalance should be corrected

- Class Ia antiarrhythmic agents such as quinidine, disopyramide.
- Class III antiarrhythmic agents such as amiodarone, sotalol.
- Other medications such as pimozone, haloperidol; imipramine antidepressants; cisapride, thioridazine, IV erythromycin, pentamidine.

Pregnancy and Lactation

Pregnancy:

Reproduction studies were performed in mouse, rat and rabbit by oral and subcutaneous route.

In the rat, fertility and reproductive performance were affected with dose related effect from 40mg/kg. This effect reversed after 4 weeks. A delay in delivery was observed at

640mg/kg by oral route and from 80mg/kg by subcutaneous route. These effects are considered to be in relation with the pharmacological activity on prolactin secretion. In none of the 3 species did the compound show embryotoxic or teratogenic effect at doses of up to 640 mg/kg.

In humans, even though in almost all cases of foetal or neonatal disorders reported in the context of sulpiride during pregnancy alternative explanations can be suggested and seem more likely, the use of sulpiride is not recommended during pregnancy because of the limited experience.

Lactation: Sulpiride has been found in the breast milk of treated women. Therefore breast-feeding is not recommended during treatment.

Neonates exposed to antipsychotic drugs during the third trimester of pregnancy are at risk for extrapyramidal and/or withdrawal symptoms following delivery. There have been reports of agitation, hypertonia, hypotonia, tremor, somnolence, respiratory distress, and feeding disorder in these neonates. These complications have varied in severity; while in some cases symptoms have been self-limited, in other cases neonates have required intensive care unit support and prolonged hospitalization.

Sulpin should be used during pregnancy only if the potential benefit justifies the potential risk to the foetus.

Side Effects

Cardiovascular disorders:

- Postural hypotension
- Cases of QT prolongation and of ventricular arrhythmias such as torsade de pointes and ventricular tachycardia, which may result in ventricular fibrillation or cardiac arrest, sudden death.

Endocrine disorders:

- Hyperprolactinaemia

General disorders and administration site conditions:

- As with all neuroleptics, malignant syndrome which is a potentially fatal complication
- Weight gain

Hepatobiliary disorders:

- Very rare cases of increase in hepatic enzymes

Nervous system disorders:

- Sedation or drowsiness. Insomnia has been reported.
- Extrapyramidal symptoms and related disorders:
- parkinsonism and related symptoms: tremor, hypertonia, hypokinesia, hypersalivation
- acute dyskinesia and dystonia (spasm torticollis, oculogyric crisis, trismus)
- akathisia

These symptoms are generally reversible upon administration of antiparkinsonian medication.

- Tardive dyskinesia (characterised by rhythmic, involuntary movements primarily of the tongue and/or the face) have been reported, as with all neuroleptics, after a neuroleptic administration of more than 3 months. Antiparkinsonian medication is ineffective or may induce aggravation of the symptoms.
- Very rare cases of convulsions have been reported in particular in patients with epilepsy.

Reproductive system and breast disorders:

Disorders related to hyperprolactinaemia:

- Galactorrhoea
- Amenorrhoea
- Gynaecomastia
- Breast enlargement and breast pain
- Orgasmic dysfunction and impotence

Skin and subcutaneous tissue disorders:

- Very rare cases of maculo-papular rash

Symptoms and Treatment of Overdose

Overdoses have been reported ranging from 1g to 16g but no death has occurred even at the 16g dose. Doses of 1g to 3g have been reported to produce restlessness and clouding of consciousness and (rarely) extrapyramidal symptoms. Doses of 3g to 7g may produce a degree of agitation, confusion, low blood pressure and extrapyramidal symptoms. In cases where coma has occurred, Sulpiride has been administered in conjunction with other psychotropic medication. The duration of intoxication is generally short, the symptoms disappearing within a few hours. There are no specific complications from overdose. In particular no haematological or hepatic toxicity has been reported.

Overdose may be treated with alkaline osmotic diuresis and if necessary, anti-parkinsonian drugs. Emetic drugs are unlikely to be effective. Coma needs appropriate nursing.

Storage Condition

Store below 30°C. Keep out of reach of children.

Presentation

10 tablets in PVC/foil blister strip. 50, 100 or 500 tablets are packed in a folded paper box.

Shelf Life

This product can be used within 36 months from the date of manufacture if kept as recommended.

Registration number

SULPIN F.C. TABLETS 200mg – MAL08061576AZ

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